

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 33

Age: 55 years old | Sex: Male

MRN: 000-TEST-33

Admission Date: 06/15/2026 | Discharge Date: 06/19/2026

Attending Physician: Dr. M. Richardson, MD | Orthopedic Surgery

****PRIMARY DIAGNOSIS & COMORBIDITIES****

- Osteoarthritis, right hip — primary reason for admission
- Status post total hip arthroplasty (THA) right hip, cemented prosthesis
- Type 2 diabetes mellitus (controlled, A1c 6.8%)
- Hyperlipidemia on statin therapy
- Hypertension (stable on current regimen)

****HOSPITAL COURSE****

Patient 33 was admitted 06/15/2026 for elective right THA due to advanced hip OA refractory to conservative management. Preop labs and imaging unremarkable. Procedure performed 06/15/2026 under spinal anesthesia with MAC; cemented acetabular and femoral components placed without intraoperative complications. EBL ~350 mL. Patient tolerated procedure well.

Postoperative course: straightforward. Hemovac drain removed POD#2 with minimal drainage. Pain controlled adequately on current regimen. Patient achieved ambulation with walker POD#1; progressed to crutches POD#3. PT/OT consulted and actively engaged in

mobility training and ADL retraining. No evidence of DVT on clinical exam. SpO2 remained >94% throughout stay. Wound healing well, no signs of infection or dehiscence.

****Key Events:****

- Foley catheter discontinued POD#2; voiding independently
- Incentive spirometry used q4h; lung sounds clear bilaterally
- CXR (06/17/2026) negative for infiltrate
- Glucose monitoring BID (fasting range 118–142 mg/dL)

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
Enoxaparin (Lovenox)	40 mg	SC daily	14 days
Metoprolol tartrate	50 mg	BID	Ongoing
Atorvastatin	20 mg	q.d. at bedtime	Ongoing
Metformin	500 mg	BID with meals	Ongoing
Oxycodone-acetaminophen	5 mg / 325 mg	PO q4–6h PRN pain	10 days
Docusate sodium (Colace)	100 mg	BID	2 weeks
Aspirin	81 mg	q.d.	Ongoing (cardioprotection)

****WEIGHT-BEARING STATUS & HIP PRECAUTIONS****

****Ambulation:**** TTWB (toe-touch weight-bearing) right lower extremity × 2 weeks, then reassess for PWB progression based on pain/tolerance and PT evaluation.

****CRITICAL Hip Precautions — Must maintain:****

- NO hip flexion >90°. Avoid bending at waist.
- NO internal rotation of hip.
- Use elevated toilet seat and shower chair (do not sit on standard furniture).
- Abductor pillow between legs when sitting or lying supine.
- Sleep supine or on left side; avoid right sidelying.
- Do NOT cross legs.

****ACTIVITY & RESTRICTIONS****

- No driving × 6 weeks (or until cleared by surgeon).
- No heavy lifting (>5 lbs) or pushing/pulling × 8 weeks.
- Avoid stairs initially